

FEATURE ARTICLE

Burnout and job satisfaction: A comparative study of psychiatric nurses from forensic and a mainstream mental health service

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ABSTRACT: Within the nursing profession stress and burnout are considered to be widely present and problematic. These factors tend to impact negatively on job satisfaction and ultimately affect the retention of nurses. Psychiatric/mental health nursing as a specialty is considered to be a highly stressful environment; however, there is a paucity of research in this area. The current study adopted a survey design to compare forensic psychiatric nurses ($n = 51$) with psychiatric nurses from a mainstreamed mental health service ($n = 78$) in relation to burnout and job satisfaction. Forensic nurses displayed lower burnout and higher job satisfaction than their counterparts from the mainstreamed services. These findings are surprising in light of the image of forensic psychiatric nursing as dangerous and unpredictable.

KEY WORDS: burnout, forensic, job satisfaction, mainstream, mental health nursing, psychiatric nursing.

INTRODUCTION

Stress, burnout and job satisfaction have become regular topics of discussion for the nursing profession in recent years. Over the past 2 years in particular, growing concern about stress and burnout in nursing has received prominence (Fagin *et al.* 1995; Gray-Toft & Anderson 1981; Hipwell *et al.* 1989; Nolan 1995) and this remains current today (Kipping 2000). The work of nursing is frequently stressful and the negative consequences experienced by nurses in carrying out their day-to-day tasks are often not fully acknowledged (Woods 1992). Stress has been identified as one of the major reasons why nurses fail to function at an optimum level of effectiveness (Kendrick 2000; Maslach & Jackson 1981; Sullivan 1993).

The concept of 'burnout' has also attracted a considerable

interest within the nursing field, despite the fact that there is no universally agreed definition (Farber 1983). The term burnout was originally used to describe the emotional exhaustion of workers in the public service (Freudenberger 1974). Maslach *et al.* (1996) were among the first researchers to explore this concept. They described it as a syndrome consisting of emotional exhaustion, depersonalization and reduced personal accomplishment. In an earlier writing, Maslach (1982) identified the strain of dealing with clients face to face as one of the most important antecedents of burnout.

Burnout is primarily caused by organizational factors and tends to be a process rather than a fixed state (Burish 1993; Miller *et al.* 1990; Schulz *et al.* 1995; Soderfeldt *et al.* 1997). It is a negative experience and results from the interaction between the individual and the environment (Sullivan 1993). Melchior *et al.* (1997) concluded that role conflict is one of the most important variables to predict burnout in psychiatric nurses. Some authors have attempted to connect burnout to individual variables, such as depression, anxiety and vulnerability (Corrigan *et al.* 1994; Piedmont 1993). McCarthy (1985) stated that nurses are particularly prone to burnout because of their lack of preparation for coping with emotional stress.

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Despite the strength of discourse and debate in relation to stress and burnout in psychiatric nursing, limited research has been conducted in this area. Carson *et al.* (1991) conducted a study with a sample of 61 community psychiatric nurses and found that 24% were experiencing emotional exhaustion and 81% had low depersonalization scores. Schafer (1992) studied 16 community psychiatric nurses using General Health Questionnaire (GHQ)-28 and Maslach Burnout Inventory (MBI) and recorded an increase in emotional exhaustion and depersonalization scores. Prosser *et al.* (1996) investigated stress and job satisfaction among hospital- and community-based mental health staff in three geographical mental health sectors in South London. The sample included 71 nurses, 23 psychiatrists, nine occupational therapists, nine nursing assistants, six social workers and three psychologists. They found that community staff scored significantly higher on the GHQ-12 and the 'emotional exhaustion' component of the MBI than hospital-based inpatient, day care or outpatient staff.

The forensic setting is frequently singled out as a particularly stressful area of nursing (Kirby & Pollock 1995). Forensic psychiatric nurses work with patients who have generally demonstrated some aggressive behaviour associated with their psychiatric disorder. The perceived threat of, and actual, physical violence is believed to contribute to high levels of stress (Coldwell & Naismith 1989; Sullivan 1993). Despite this view, little research has investigated the actual incidence of stress and/or burnout in forensic psychiatric nurses, particularly in comparison with psychiatric nurses employed in non-forensic settings. A study (Coffey 1999; Coffey & Coleman 2000) of 104 forensic community mental health nurses in the UK demonstrated a higher burnout among forensic community mental health nurses who are working in the National Health Service. The sample comprised all forensic community mental health nurses ($n = 104$) attached to the 26 National Health Service Medium Secure Units in England and Wales.

The above study investigated the level of stress in forensic community mental health nurses using MBI, GHQ-28 and Community Psychiatric Nurse Stress Questionnaire (CPNSQ) and found that a number of respondents were experiencing burnout. The findings suggested that a substantial portion (44.3%) of forensic mental health nurses were experiencing high burnout in relation to emotional exhaustion. They also found a statistically significant association between caseload size and level of stress.

Job satisfaction has been described as an affective state that depends on the interaction of employees, their personal characteristics, values and expectations with the work environment and the organization (Mueller & McCloskey 1990). Job dissatisfaction is often construed as a source of and an

outcome measure of stress and burnout (Cooper & Baglioni 1988). Early studies have shown an inverse relationship between job satisfaction and burnout; for example, higher job satisfaction tends to be accompanied by lower levels of burnout (Dolan 1987; Pines *et al.* 1981).

Research evidence consistently shows that structure, organizational atmosphere, job tasks, pay, potential for advancement, personal recognition, leadership style and leadership effectiveness are the major factors to impact on job satisfaction in nursing (Gillies *et al.* 1990; Mansen 1993; Wells 1990). Oermann and Bizek (1994) found that organization of the work environment, autonomy, involvement in decision-making, interpersonal relationships and recognition from others influenced job satisfaction. Blegen (1993) completed a meta-analysis of 48 studies on job satisfaction in nursing and concluded that job satisfaction is very strongly related to stress (negatively) and commitment to the organization (positively).

There is a paucity of research in relation to job satisfaction in the specific field of psychiatric nursing and the studies demonstrate inconsistent findings. Parahoo (1991) investigated the perception of community psychiatric nurses' level of job satisfaction and the factors they identified as contributing to their satisfaction or dissatisfaction with the job in the Northern Ireland. She found that 54 out of 77 community psychiatric nurses (70.1%) rated their satisfaction 'high' or 'very high', while only 4% reported a low level of satisfaction. Onyett *et al.* (1997) studied emotional exhaustion, low personal accomplishment, depersonalization, job satisfaction and sick leave among 445 Community Mental Health Team (CMHT) members in England. They reported high emotional exhaustion among consultant psychiatrists, social workers, nurses and psychologists. High job satisfaction, higher personal accomplishment and low depersonalization were also found. Job satisfaction was associated with team role clarity and identification with the team. Caseload size, composition and the frequency with which service users were seen were not associated with job satisfaction or burnout.

A study by Hannigan *et al.* (2000) found similar results regarding burnout in community mental health nurses. Hannigan *et al.* (2000) investigated the causes, moderators and outcomes of stress among 301 community mental health nurses in Wales. Of these, 283 completed the MBI. They found that half of the respondents ($n = 145$, 51%) were emotionally overextended and exhausted by their work. One-quarter of respondents were found to possess negative attitudes towards their clients and approximately one in seven experienced little or no sense of satisfaction with their work.

Clinton and Hazelton (2000), in a recent Australian study, found that mental health nurses report high levels of stress compared to nurses from other practice areas.

'Stress and burnout arise from the pace of change in mental health services, the perception that the personal safety of the nurse is under threat in acute units in the community, and from the perceived over-bureaucratization of mental health services' (p. 160). Although the prevalence of burnout and work-related stress in psychiatric nurses has been established, the causes have not yet been identified. The paucity of literature exploring the concepts of stress, burnout and job satisfaction and their impact on psychiatric nurses, particularly in Australia, clearly indicates the need for research in this area.

The results presented in the present paper form part of a study with the overall aim of measuring the effectiveness of clinical supervision in reducing stress and burnout in psychiatric/mental health nursing. This paper is concerned with reporting the levels of stress, burnout and job satisfaction in psychiatric/mental health nurses employed in a forensic in comparison to a mainstream mental health service.

METHODS

Sample

The survey instruments were distributed to 95 forensic psychiatric nurses and 96 psychiatric nurses employed in a mainstream mental health service in the Melbourne metropolitan area. Both samples were drawn from nurses across all practice settings including inpatient and community and, in the case of the forensic nurses, also included nurses employed within the prison and court systems.

The survey was distributed by hand with an information sheet and included MBI (Maslach *et al.* 1996), Job Satisfaction Scale (JSS) of Nurse Stress Index (Harris *et al.* 1998) and Satisfaction with Nursing Care and Work (SNCW) (Hallberg *et al.* 1994). The present study was conducted as part of a larger study on the impact of clinical supervision on stress and job satisfaction in psychiatric nursing. Completed questionnaires were received from 51 forensic nurses (return rate = 54%) and 78 mainstream psychiatric nurses (return rate = 81%).

Data analysis

The Statistical Package for Social Sciences (SPSS) was used to analyse the data. The initial analysis was conducted by calculating frequencies, mean scores and standard deviations. Two-tailed *t*-tests for independent groups: forensic and

mainstream psychiatric nurses, were used to calculate the difference between the two groups and to test for relationships between elements of burnout, and satisfaction with nursing care and work as measured on MBI and SNCW.

Measures

The validity and reliability of the research instruments used in the present study have been demonstrated as effective in the measurement of stress, burnout and job satisfaction: MBI (Pierce & Molloy 1989), JSS (Gray-Toft & Anderson 1981) and SNCW (Hallberg *et al.* 1994).

Maslach Burnout Inventory

The MBI is a 22-item scale designed to measure three aspects of burnout: emotional exhaustion, depersonalization and personal accomplishment. For emotional exhaustion and depersonalization subscales, high mean scores reflect high level of burnout while for the personal accomplishment subscale, high scores reflect low level of burnout. (Maslach & Jackson 1986). Respondents were asked to rate items on a scale from 0, 'never' to 6 'everyday' and mean scores were obtained for each of the three subscales from these frequencies. The higher the score on the emotional exhaustion and the depersonalization subscales the greater the degree of burnout. The low scores on the personal accomplishment reflect a high degree of burnout. The normative scores of MBI are presented in Table 1.

Job Satisfaction Scale of the Nurse Stress Index

The JSS is a five-item questionnaire with five-point response scales (1 = strongly disagree, 2 = disagree, 3 = undecided, 4 = agree and 5 = strongly agree) designed to measure the job satisfaction. The higher the total score, the greater the level of job satisfaction.

Satisfaction with Nursing Care and Work

The SNCW comprises 33 statements regarding atmosphere, dissatisfaction, flexibility, commitment and involvement in nursing care and work. The response format ranges from 'fully agree' (1) to 'strongly disagree' (5). The lower the scores, the greater the satisfaction with nursing care and work (Berg & Hallberg 1999). The subcategory of 'satisfactory participation and implementation of individualized care improves care quality' in the original questionnaire was changed into 'satisfactory participation and clinical supervision improves care quality' for the purposes of this study.

TABLE 1: Interpreting scores for the Maslach Burnout Inventory for psychiatric nurses from forensic and mainstream mental health services

Subscale	Normative low	Normative medium	Normative high
Emotional exhaustion	<13	14–20	>20
Depersonalization	<4	5–7	>8
Personal accomplishment	>34	33–29	<28

RESULTS

Burnout

Maslach Burnout Inventory

Analysis of the mean scores on the emotional exhaustion (EE) subscale was 12.9 (SD 7.5) for forensic nurses compared to 17.4 (SD 12.2) for psychiatric nurses employed in mainstream services, demonstrating low levels of burnout for forensic nurses and medium or moderate burnout for psychiatric nurses employed in mainstream services. Mean scores for the depersonalization (DP) subscale were similar at 4.7 (SD = 6) for both groups of nurses, indicating moderate levels of depersonalization. On the personal accomplishment (PA) subscale, the mean scores indicated low levels of burnout for both forensic nurses and psychiatric nurses employed in mainstream services. Although some differences were evident between the two groups these were not found to be statistically significant. These results are presented in Table 2.

Descriptive data relating to the different categories of burnout: 'high burnout', 'moderate burnout' and 'low burnout' are presented in Table 3. More psychiatric nurses employed in mainstream services suffer 'higher' burnout on the emotional exhaustion subscale when compared with forensic nurses (35.8% *vs* 15.6%). Similarly, more forensic nurses record low burnout. On the depersonalization subscale, more psychiatric nurses from mainstream services record 'high burnout' in comparison with forensic nurses (24.3% *vs* 17.6%). The percentage of both types of nurses who experience 'moderate burnout' on the emotional exhaustion and the depersonalization scores is

similar. On the personal accomplishment subscale, more mainstream psychiatric nurses record a 'high' score than do forensic nurses (23% *vs* 17.6%). However, both types of nurses recorded a higher percentage of low scores on the personal accomplishment subscale.

Job satisfaction

Job Satisfaction Scale

The scores obtained from the JSS suggest that forensic psychiatric nurses are more satisfied with their current situation at work than the nurses from mainstream services. In particular, forensic psychiatric nurses indicate greater satisfaction with their present level of involvement in decision-making at work and in the degree of support they receive in their job. Responses suggest that forensic nurses are less likely to consider finding another job within nursing. However, a higher proportion of forensic nurses considered finding an occupation other than nursing compared with their colleagues from the mainstream service. This information is presented in Tables 4 and 5.

Satisfaction with Nursing Care and Work

Mean scores on the SNCW Scale are presented in Tables 6 and 7. Statistically significant differences in scores between forensic psychiatric nurses and psychiatric nurses employed in the mainstreamed facility were detected in only one of the categories. Forensic nurses were more satisfied in the subcategory of 'satisfactory co-operation and comfort'.

Table 8 provides a descriptive analysis of both groups of nurses' satisfaction with nursing care and work under eight main categories. Almost all forensic nurses agreed that nursing provides engaging and stimulating tasks, it

TABLE 2: Results of the Maslach Burnout Inventory for psychiatric nurses from forensic and mainstream mental health services

Forensic unit (<i>n</i> = 51)				Mainstream mental health service (<i>n</i> = 78)		
Maslach Burnout Inventory						
Subcategories	<i>N</i>	Mean	SD	<i>N</i>	Mean	SD
Emotional exhaustion	51	12.9	7.5	78	17.4	12.2
Depersonalization	51	4.7	6	78	4.5	4.9
Personal accomplishment	51	34.5	7.9	78	35.6	9.8

TABLE 3: Summary of percentages of respondents in high, moderate and low burnout categories of the Maslach Burnout Inventory

Subscale	Mainstream mental health service			Forensic mental health unit		
	High scores	Moderate scores	Low scores	High scores	Moderate scores	Low scores
EE	n = 28 (35.8%)	n = 15 (19.2%)	n = 36 (46.1%)	n = 8 (15.6%)	n = 11 (21.5%)	n = 32 (62.7%)
DP	n = 19 (24.3%)	n = 11 (14.1%)	n = 48 (61.5%)	n = 9 (17.6%)	n = 8 (15.6%)	n = 34 (66.6%)
PA	n = 18 (23%)	n = 9 (11.5%)	n = 51 (65.3%)	n = 9 (17.6%)	n = 10 (19.6%)	n = 32 (62.7%)

EE, emotional exhaustion (low = <13, moderate = 14–20, high = >21).

DP, depersonalization (low = <4, moderate = 5–7, high = >8).

PA, personal accomplishment (low = >34, moderate = 33–29, high = <28).

TABLE 4: *Job satisfaction of psychiatric nurses from mainstream mental health services*

Statement	Mainstream mental health service (n = 78)						
	Strongly disagree (%)	Disagree (%)	Disagree-ment (%)	Undecided (%)	Agree (%)	Strongly agree (%)	Agree-ment (%)
I am satisfied with my current situation at work	4 (5.1)	11 (14.1)	15 (19.2)	13 (16.7)	43 (55.1)	7 (9)	50 (64.1)
I am satisfied with my present level of involvement in decision-making at work	5 (6.4)	12 (15.4)	17 (21.8)	11 (14.1)	41 (52.6)	9 (11.5)	50 (64.1)
I am satisfied with the degree of support I receive in my job	11 (14.1)	16 (20.3)	27 (34.4)	14 (17.9)	28 (35.4)	9 (11.4)	37 (46.8)
I seldom think about finding another job within nursing	6 (7.8)	24 (31.2)	30 (38)	11 (14.3)	27 (35.1)	9 (11.7)	36 (46.8)
I seldom think about finding an occupation other than nursing	13 (16.7)	19 (24.4)	32 (41.1)	10 (12.8)	21 (26.9)	15 (19.2)	36 (46.1)

TABLE 5: *Job satisfaction of psychiatric nurses from forensic psychiatric services*

Statement	Forensic psychiatric services (n = 51)						
	Strongly disagree (%)	Disagree (%)	Disagree-ment (%)	Undecided (%)	Agree (%)	Strongly agree (%)	Agree-ment (%)
I am satisfied with my current situation at work	2 (3.9)	2 (3.9)	4 (7.8)	8 (15.7)	27 (52.9)	12 (23.5)	39 (76.4)
I am satisfied with my present level of involvement in decision-making at work	3 (6%)	3 (6)	6 (12)	5 (10)	31 (62)	8 (16)	39 (78)
I am satisfied with the degree of support I receive in my job	2 (3.9)	3 (5.8)	5 (9.7)	8 (15.7)	25 (49)	13 (25.5)	38 (74.5)
I seldom think about finding another job within nursing	4 (7.8)	10 (19.6)	14 (27.4)	7 (13.7)	17 (33.3)	13 (25.5)	30 (58.8)
I seldom think about finding an occupation other than nursing	3 (5.9)	10 (19.6)	13 (25.5)	11 (21.6)	16 (31.4)	11 (21.6)	27 (53)

TABLE 6: *Satisfaction with nursing care and work (SNCW) of psychiatric nurses from forensic psychiatric services*

Subcategory	Forensic psychiatric nurses (n = 51)	
	Mean	SD
Professional growth	2.2	0.72
Satisfactory autonomy and care quality	2.06	0.78
Information to patients	2.8	0.82
Satisfactory cooperation and comfort	2.06	1.1
Satisfactory participation and clinical supervision improves care quality	1.5	0.94
Emotional development	2.6	1.04
Unsatisfactory knowledge about patients and work	3.4	0.77
Positive attitudes to family participation and empathy important	2.4	0.86

TABLE 7: *Satisfaction with nursing care and work (SNCW) of psychiatric nurses from mainstream mental health services*

Subcategory	Mainstream psychiatric nurses (n = 78)	
	Mean	SD
Professional growth	2.3	0.98
Satisfactory autonomy and care quality	2.2	0.97
Information to patients	2.6	0.94
Satisfactory cooperation and comfort	2.4	1.1
Satisfactory participation and clinical supervision improves care quality	2.04	0.91
Emotional development	3.2	1.03
Unsatisfactory knowledge about patients and work	3.0	0.99
Positive attitudes to family participation and empathy important	2.3	0.93

TABLE 8: *Satisfaction with nursing care and work (SNCW) of psychiatric nurses from forensic and mainstream mental health services*

	n	Forensic		n	Mainstream mental health service	
		Agree (%)	Disagree (%)		Agree (%)	Disagree (%)
Professional growth						
Nursing provides engaging and stimulating tasks	51	51 (100%)	0	76	73 (96)	3 (4)
Nursing provides good prospects for professional development	51	50 (98)	1 (1.9)	74	57 (77)	17 (23)
Nursing provides varying tasks	51	51 (100)	0	77	72 (93.5)	5 (6.5)
Nursing provides good prospects for personal development	51	51 (100)	0	76	67 (88.1)	9 (11.9)
Lack of time at work	51	27 (52.9)	24 (47)	74	48 (64.8)	26 (35.2)
Satisfactory autonomy and care quality						
I am satisfied with my responsibility	51	47 (92.1)	4 (7.8)	76	62 (81.5)	14 (18.5)
I am satisfied with my independence	51	49 (96)	2 (3.9)	76	71 (93.4)	5 (6.4)
I am satisfied with my care organization	49	49 (100)	0	74	65 (87.8)	9 (12.2)
I am satisfied with my quality of care	50	50 (100)	0	77	74 (96.1)	3 (3.7)
I am satisfied with opportunity to discuss work	51	47 (92.1)	4 (7.8)	76	60 (78.9)	16 (21.1)
Information to patients						
I am satisfied with information to patients about disease	50	43 (86)	7 (14)	75	12 (16)	63 (84)
I am satisfied with information provided before treatment and examination	50	48 (96)	2 (4)	74	57 (77)	17 (23)
I am satisfied with information provided to newly admitted patient about ward	50	49 (98)	1 (2)	72	62 (86.1)	10 (13.9)
I am satisfied with information provided to families about care	50	42 (84)	8 (16)	77	59 (76.6)	18 (23.4)
Satisfactory co-operation and comfort						
I am satisfied with friendly atmosphere	51	49 (98)	2 (2)	77	65 (84.4)	12 (15.6)
I am satisfied with co-operation	51	50 (98)	1 (2)	75	63 (84)	12 (16)
I am satisfied with comfort at work	50	46 (92)	4 (8)	77	63 (81.8)	14 (22.2)
Satisfactory participation and clinical supervision improves care quality						
Clinical supervision increases quality of care	50	48 (96)	2 (4)	74	71 (95.9)	3 (4.1)
Clinical supervision increases stimulation at work	50	48 (96)	2 (4)	74	70 (94.5)	4 (5.5)
Supervision may influence pace	49	47 (95.9)	2 (4)	71	64 (90.1)	7 (9.9)
Regular supervision is welcome	51	49 (98)	2 (4)	74	70 (94.5)	4 (5.5)

(continued)

TABLE 8: *Satisfaction with nursing care and work (SNCW) of psychiatric nurses from forensic and mainstream mental health services (continued)*

	Forensic			Mainstream mental health service		
	<i>n</i>	Agree (%)	Disagree (%)	<i>n</i>	Agree (%)	Disagree (%)
Emotional development						
My working hours intrude on my private life	50	36 (72)	14 (28)	76	38 (50)	38 (50)
It is boring to care for the same patient	50	19 (38)	31 (62)	75	23 (30.6)	52 (69.4)
Too much involvement wears you down	51	38 (74.5)	13 (25.5)	75	47 (62.6)	28 (37.4)
Little time to penetrate patients' feelings	49	28 (57)	21 (43)	76	50 (65.7)	26 (34.3)
Too much to expect involvement in each patient	49	26 (53)	23 (47)	75	39 (52)	36 (48)
Unsatisfactory knowledge about patients and work						
I feel I know too little about patients' life story, needs and wishes	50	21 (42)	29 (58)	75	40 (53.3)	35 (46.7)
I feel I know too little about patients' disease and treatment	50	9 (18)	41 (82)	76	26 (34.2)	50 (65.8)
I feel that I have no right to be angry with patients	50	16 (32)	34 (68)	76	42 (55.2)	34 (44.8)
I am worried about future working conditions	51	31 (60.7)	20 (39.3)	76	62 (81.5)	14 (18.5)
Positive attitudes to family participation and empathy important						
Positive attitudes to families taking part in care	50	48 (96)	2 (4)	76	75 (98.6)	1 (1.4)
It is important that I can take in patients' feelings	50	46 (92)	4 (8)	75	71 (94.6)	4 (5.4)
I feel that all staff members ought to do all tasks	51	17 (34)	34 (66)	77	43 (55.8)	34 (44.2)

provides good prospects for professional development and it provides good prospects for personal development. In comparison, of the psychiatric nurses employed in the mainstreamed facility, 77% felt that nursing provides good prospects for professional development and 88.1% felt that nursing provides good prospects for personal development. More mainstream psychiatric nurses considered lack of time at work as a concern than the forensic psychiatric nurses.

In the Satisfactory autonomy and care quality subcategory, forensic nurses are more satisfied with their responsibility, their independence, the care organization, their quality of care and the opportunity to discuss work than the psychiatric nurses employed in the mainstreamed facility. In the category, Information to patients, 86% of forensic nurses were satisfied with information to patients about disease as opposed to only 16% of nurses employed in the mainstreamed facility. Forensic nurses showed a higher degree of satisfaction with information provided to a newly admitted patient about the ward, and information provided to families about care than their counterparts from the mainstreamed service. In the category, Satisfactory co-operation and comfort, forensic psychiatric nurses showed a higher level of satisfaction with friendly atmosphere, co-operation and comfort at work, than the mainstream psychiatric nurses. Similarly, mainstream

psychiatric nurses agreed that they had little knowledge about the disease and treatment, and needs and wishes of their patients than their forensic counterparts. The nurses employed in mainstream services appear more likely to be worried about working conditions in the future.

Both groups of nurses strongly endorsed the value of clinical supervision to the quality of care and to their own sense of satisfaction and indicated an acceptance of the notion of regular supervision.

The psychiatric nurses working in the mainstreamed service indicated less knowledge about the patients' life stories, needs and wishes, and about patients' diseases and treatment than forensic nurses. Interestingly, forensic nurses were less likely to consider that they have no right to be angry with patients. Both groups of nurses held positive attitudes to families taking part in care.

DISCUSSION

The nurses employed within the mainstreamed mental health service demonstrated a higher level of burnout than the forensic psychiatric nurses on the MBI, specifically in the areas of emotional exhaustion and depersonalization. Similarly, the forensic nurses recorded a higher level of job satisfaction than their mainstream counterparts. More specifically, they indicated greater satisfaction with their

current situation at work and involvement in decision-making, and more satisfaction with the amount of support they receive.

Mean scores on job satisfaction measured by the SNCW showed that, overall, both the forensic and the mainstreamed psychiatric nurses are essentially similar in terms of satisfaction with nursing care and work. When the individual subcategories of the scale were compared, there was a statistically significant difference to suggest that forensic nurses are more satisfied than mainstreamed nurses in the 'Satisfactory co-operation and comfort' subcategory. However, when these subcategories are reduced to single items (statements) (Table 8), forensic nurses appear to be markedly more comfortable with their nursing care and work than their mainstreamed counterparts. For example, 86% of forensic nurses are satisfied with information to patients about their disease whereas only 16% of mainstreamed nurses are satisfied with the same item.

Mean scores on all three scales seem to suggest that, overall, the forensic psychiatric nurses are 'less' burned out and 'more' satisfied, whereas the mainstreamed psychiatric nurses are 'more' burned out and 'less' satisfied with their profession, work environment, peer support and professional growth. This finding is concerning and may have worrying implications not only for patient outcomes but also for nursing recruitment and retention.

These results are at variance with the popular misconception that forensic psychiatry is a more stressful area of practice, involving the care of more dangerous and unpredictable patients. In keeping with this image, one would expect forensic nurses to be more stressed and burned out than the psychiatric nurses from mainstreamed services. Given the results of this study, further investigation is warranted to gain a deeper understanding of why the forensic environment appears to be more positive and supportive than the environment of the mainstreamed service.

These results must, however, be viewed with a degree of caution. While the forensic sample comprises a significant proportion of forensic psychiatric nurses in Victoria, the number of nurses from the mainstreamed service constitutes a very small proportion of mainstream psychiatric nurses in Victoria. Furthermore, as they are all employed within the one mental health service, it is not possible to conclude the extent to which their responses are specific to the individual service, rather than to psychiatric nursing within mainstream services at a more general level. A further limitation may be seen in the fact that the while nurses surveyed represent a cross-section of service types including inpatient and community, this information was not collected (largely due to the small overall sample size). It is, therefore, not possible to determine the impact of the type of service setting (e.g. inpatient or community) on the level of burnout and job satisfaction experienced.

Nevertheless, these results are interesting and require further investigation, particularly as methods for improving recruitment are investigated.

A further interesting point worthy of further investigation is that although forensic nurses appear to be more satisfied with their work and working environment, their responses indicate that they are more likely to consider a job outside of nursing than the nurses employed within the mainstreamed facility. Further research is required to examine the extent to which this difference is indicative of dissatisfaction with nursing as a profession.

As mental health services continue to grapple with the problems of retention of nursing staff, more than half of the total responses to this survey indicate a willingness to leave the profession should an opportunity arise elsewhere. These results, therefore, lend further support to the urgent need to probe and consider the factors influencing psychiatric nurses' satisfaction, and the identification of strategies to overcome or, at the very least, to minimize these factors.

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